

Input Form (IF)

Score: 2/5 — Significant gaps | Confidence: high

Benchmark: SAMSUM | Context: South Asia and MENA — Clinical NLP Researchers and AI Practitioners

Input Form definition: Whether the input signal encoding (text, audio, image parameters) matches deployment conditions.

Each section below is followed by an evidence table. Three types of evidence are cited: **[QN]** — verbatim quotes extracted from the benchmark paper; **[WEB-N]** — web sources consulted during assessment; **[DN]** / **[DATASET-DN]** — sampled datapoints from the benchmark dataset, with an interpretation note.

Justification

The plain-text turn-by-turn speaker-attributed format [\[Q29, Q33\]](#) is structurally compatible with clinical NLP pipelines and is empirically clean and consistent ([DATASET-D17](#), [DATASET-D19](#)). Speaker attribution and utterance separators have been shown to benefit models [\[Q65, Q66\]](#). However, all formatting decisions, tokenization, and length calibrations [\[Q42\]](#) assume English plain text exclusively. There is no provision for Arabic RTL rendering, Urdu Nastaliq, Devanagari, morphologically rich tokenization, or cross-script code-switching common in South Asian clinical contexts. Empirical sampling confirms 100% English Latin-script content with no non-Latin or RTL examples ([DATASET-D17](#), [DATASET-D19](#)). The MODERATE priority weight reflects partial transferability of structural conventions with substantive multilingual mismatches.

ID	Evidence
Q29	"Beforehand, we specified a format for written dialogues with summaries: a colon should separate an author of utterance from its content, each utterance is expected to be in a separate line." (p.2)
Q33	"Each utterance contains the name of the speaker." (p.3)
Q42	"In all our experiments, news and dialogues are truncated to 400 tokens, and summaries – to 100 tokens." (p.4)
Q65	"On the other hand, dialogues are divided into utterances, and for each utterance its author is assigned." (p.7)
Q66	"We demonstrate in experiments that the models benefit from the introduction of separators, which mark utterances for each person." (p.7)
D17	Eva: Meeting today with clients at 11 am sharp.. / Jim: Yes sir / Jim: Yes sir I have — Semi-formal workplace exchange; conventional English register
D19	Tom: Did anyone pick up prints? / Glen: I didn't have time, but called John? — Minimal workplace coordination; turn-by-turn structure with speaker attribution

Strengths

- Consistent plain-text format compatible with standard NLP preprocessing pipelines ([DATASET-D17](#), [DATASET-D19](#))
- Multi-party dialogue structure with named speaker attribution exercises clinically relevant speaker-tracking [\[DATASET-D15\]](#)
- Format tolerates emoticons, kaomoji, and informal register variation without structural disruption [\[DATASET-D18\]](#)

ID	Evidence
D15	Terry: I have a question. Does anybody know what's the youngest country in the world / Kate: South Sudan — Trivia exchange — no clinical relevance
D17	Eva: Meeting today with clients at 11 am sharp.. / Jim: Yes sir / Jim: Yes sir I have — Semi-formal workplace exchange; conventional English register
D18	Ammalee: This lasted over a month.👉 (· o ·)👈👉 (· o ·)👈 / Maryann: Ah! Hello Ma'am! Thank you for the good review! — Emoticons and Unicode kaomoji used naturally in dialogue
D19	Tom: Did anyone pick up prints? / Glen: I didn't have time, but called John? — Minimal workplace coordination; turn-by-turn structure with speaker attribution

Checklist

Checklist item definitions:

ID	Assessment Question
IF-1	Compare signal distributions between source and target
IF-2	Check if regional infrastructure supports data capture specs
IF-3	Identify domain-specific form differences
IF-4	Document form mismatches harming external validity

Checklist responses:

IF-1: Signal distribution is text-only English Latin script. Target deployment requires Arabic RTL, Urdu Nastaliq, Devanagari, Bengali, and code-switched content per regional context YAML; none are present.

IF-2: Regional infrastructure supports text-based pipelines, but Arabic NLP for mental health faces unique challenges (morphological richness, dialectal variation, diacritics) per WEB-12, none of which the benchmark form addresses. Token truncation at 400 [Q42] was calibrated for English and may interact poorly with morphologically agglutinative target languages.

IF-3: Domain-specific form gaps include: no provision for code-switching markers (Urdu-English, Hindi-English clinical contexts noted in regional YAML), no script-direction handling, no diacritic preservation conventions for Arabic clinical text.

IF-4: External validity is harmed for multilingual deployment: the benchmark's input form provides no signal for non-English clinical inputs, while structural conventions (speaker attribution, turn separators) do partially transfer.

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Q66	"We demonstrate in experiments that the models benefit from the introduction of separators, which mark utterances for each person." (p.7)
WEB-12	Arabic NLP challenges include morphological richness, dialectal variation, and diacritics — unaddressed by SAMSum form (https://www.mdpi.com/2227-9032/13/9/963)

Information Gaps

- Token length distribution interactions with Arabic/Urdu morphology
- Whether placeholder tokens for media [DATASET-D18] align with clinical platform conventions

Requires Expert Verification

- Impact of English-calibrated truncation thresholds on cross-lingual fine-tuning

Remediation

Gap 1: Format conventions and tokenization are calibrated for English Latin script with no provision for RTL Arabic, Urdu Nastaliq, Devanagari, or code-switching.

Recommendation: Establish multilingual preprocessing pipelines (RTL-aware rendering, morphology-aware tokenization for Arabic and Urdu, code-switching markers) before any cross-lingual application of SAMSum-derived models. Recalibrate length thresholds for morphologically richer target languages.